

CHAPTER 3

The triage method

Introduction

The method outlined in this book is designed to allow the triage practitioner to rapidly assign a clinical priority to each patient. The system selects patients with the highest priority first without making any assumptions about the diagnosis. Emergency Departments are, to a large extent, driven by the patients' presenting signs and symptoms and this lack of focus on diagnosis is, therefore, deliberate.

Five-step process to triage decision making

- 1 Identify the problem
- 2 Gather and analyse information related to the solution
- 3 Evaluate all the alternatives and select one for implementation
- 4 Implement the selected alternative
- 5 Monitor the implementation and evaluate outcomes

Identifying the problem

Clinical practice is geared around the concept of a *presenting complaint* – that is the chief sign or symptom identified by the patient or carer. A list of presentations pertinent to triage is shown below.

Abdominal pain in adults	Irritable child
Abdominal pain in children	Limb problems
Abscesses and local infections	Limping child
Abused or neglected child	Major trauma
Allergy	Mental illness
Apparently drunk	Neck pain
Assault	Overdose and poisoning

(Continued)

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Asthma	Palpitations
Back pain	Pregnancy
Behaving strangely	PV (per vaginum) bleeding
Bites and stings	Rashes
Burns and scalds	Self-harm
Chemical exposure	Sexually acquired infection
Chest pain	Shortness of breath in adults
Collapsed adult	Shortness of breath in children
Crying baby	Sore throat
Dental problems	Testicular pain
Diabetes	Torso injury
Diarrhoea and vomiting	Unwell adult
Ear problems	Unwell baby
Eye problems	Unwell child
Facial problems	Unwell newborn
Falls	Urinary problems
Fits	Worried parent
Foreign body	Wounds
Gastrointestinal (GI) bleeding	Major incidents – primary
Headache	Major incidents – secondary
Head injury	

This list of presentational flow charts covers almost all presentations to Emergency Departments. The list, charts and contents were finalised after considerable discussion and have been refined, in both this and previous editions, following changes in practice, research and international consultation. The presentations fall broadly into the categories of illness, injury, children, abnormal and unusual behaviour, and major incidents.

The first part of the triage method requires the practitioner to select the most appropriate presentational flow chart from the list. The chart identifies discriminators that allow the clinical priority to be determined.

A key feature of the method is that the charts are consistent in their approach, since it is recognised that a number of patients' chief complaints may lead to more than one presentational flow chart. For example, a patient who presents feeling generally unwell with a stiff neck and a headache will be given the same priority whether the practitioner uses the *Unwell adult*, *Neck pain* or *Headache* flow charts.

Gathering and analysing information

To a great extent the patient's presentation will dictate which presentational flow chart is selected. Following this selection, information must be gathered and analysed to allow the actual priority to be determined. The

flow chart structures this process by showing key discriminators at each level of priority – the assessment is carried out by finding the highest level at which the answer posed by the discriminator question is positive. Discriminators are deliberately posed as questions by the triage practitioner to facilitate the process.

Discriminators

Discriminators, as their name implies, are factors that discriminate between patients, such that they allow them to be allocated to one of the five clinical priorities. They can be *general* or *specific* and are arranged in the ABCDE format. General discriminators apply to all patients irrespective of their presentation and therefore appear time and time again throughout the charts; on each occasion the general discriminators will lead the triage practitioner to allocate the same clinical priority. Specific discriminators are applicable to individual presentations or to small groups of presentations, and tend to relate to key features of particular conditions. Thus while *severe pain* is a general discriminator, *cardiac pain* and *pleuritic pain* are specific discriminators. General discriminators appear in many more charts than specific ones. All the discriminators used are defined in the discriminator dictionary at the end of the book, and the definitions of the specific ones in use on individual charts are repeated on the accompanying chart notes for ease of reference. All discriminators are reviewed for each edition. Any changes are published on the triage website at www.triagenet.net.

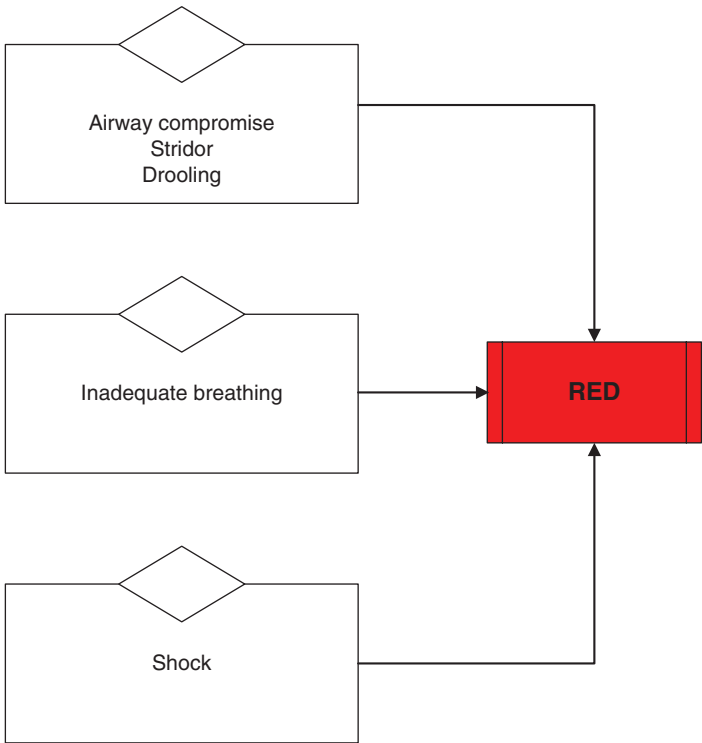
General discriminators are a recurring feature of the charts, and a proper understanding of them is essential to an understanding of the triage method. Six general discriminators are discussed further here:

- Life threat
- Conscious level
- Haemorrhage
- Temperature
- Pain
- Acuteness

Life threat

To a practising emergency nurse or emergency physician *life threat* is perhaps the most obvious general discriminator of all. Broadly speaking this recognises that any cessation or threat to the vital (ABC) functions places the patient in priority 1 (red).

Patients who are unable to maintain their own airway for any length of time have an insecure airway. Additionally, patients with stridor have significant airway threat – this may be an inspiratory or expiratory noise, or both. Stridor is heard best on breathing with the mouth open. Absence of breathing is defined as no respiration or respiratory effort as assessed by looking, listening and feeling for 10 seconds. Inadequacy is a more difficult concept – but in general, patients who are failing to breathe well enough to maintain adequate oxygenation have inadequate breathing. There may be an increased work of breathing, signs of inadequate breathing or exhaustion. Absence of pulses is only diagnosed after palpation over a central pulse for 5 seconds. Shock can be difficult to diagnose – the classic signs include sweating, pallor, tachycardia, hypotension and reduced conscious level.

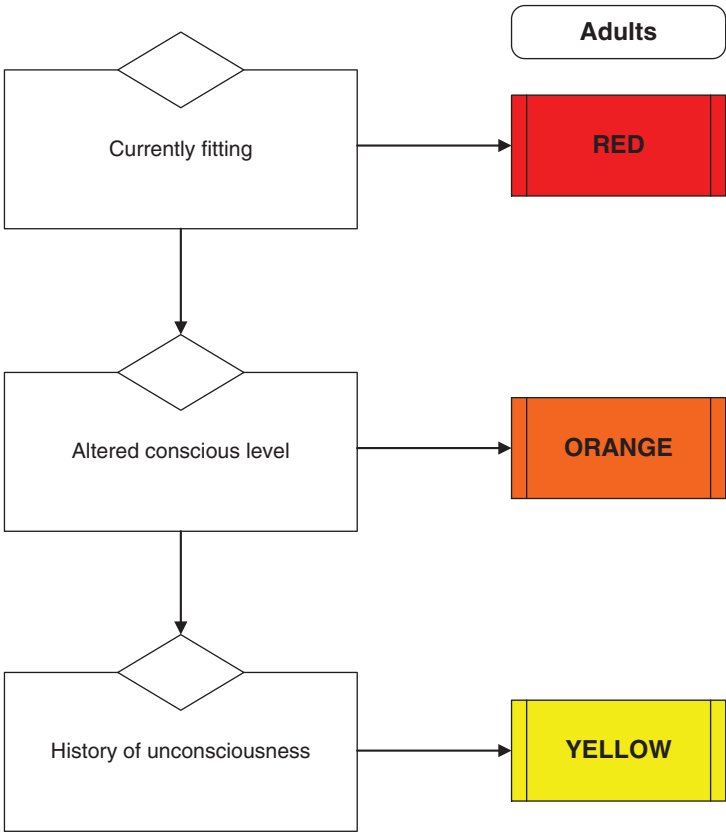


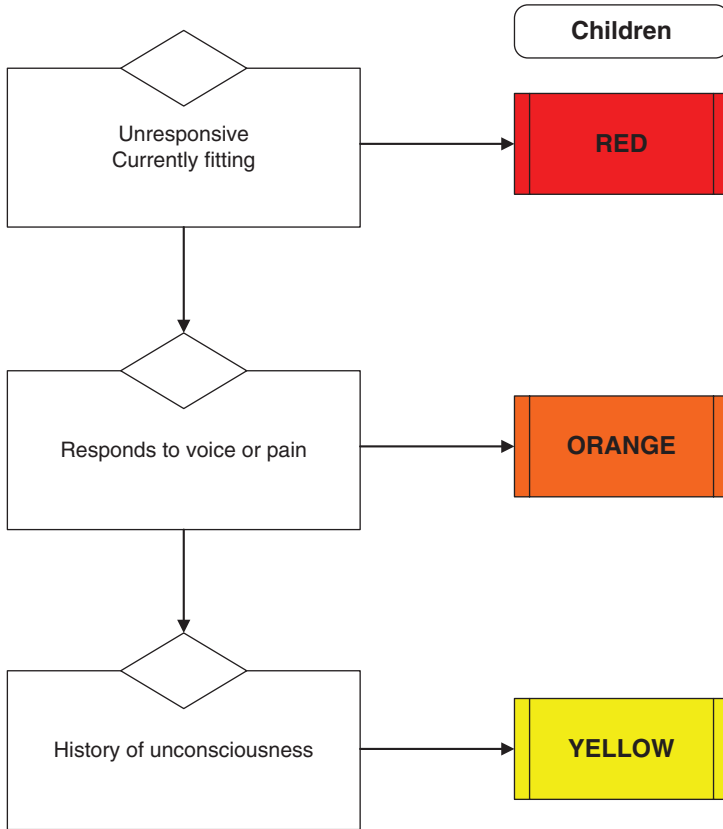
Conscious level

Conscious level is considered differently for adults and children. In adults only currently fitting patients are always categorised as priority 1 (red), while all unresponsive children are placed in this clinical priority. Adult

patients with altered conscious level (responding to voice or pain or unresponsive) are categorised as priority 2 (orange), as are children who respond to voice or pain only. All patients with a history of unconsciousness should be allocated to priority 3 (yellow).

The fact that all patients with alterations in conscious level are allocated to the very urgent priority may be at odds with current practice; this is especially so with regard to the clinical priority given to patients who are intoxicated or under the influence of drugs. Two points need to be made about this. Firstly, the aetiology of alterations in level of consciousness is largely irrelevant in determining the risk to the patient – an altered conscious level due to drugs or alcohol is clinically as important as an altered conscious level due to other causes. Secondly, most drunk patients do not have an altered level of consciousness. Specific points about the allocation of clinical priority to patients who are apparently drunk are dealt with in the presentational flow chart of that name.

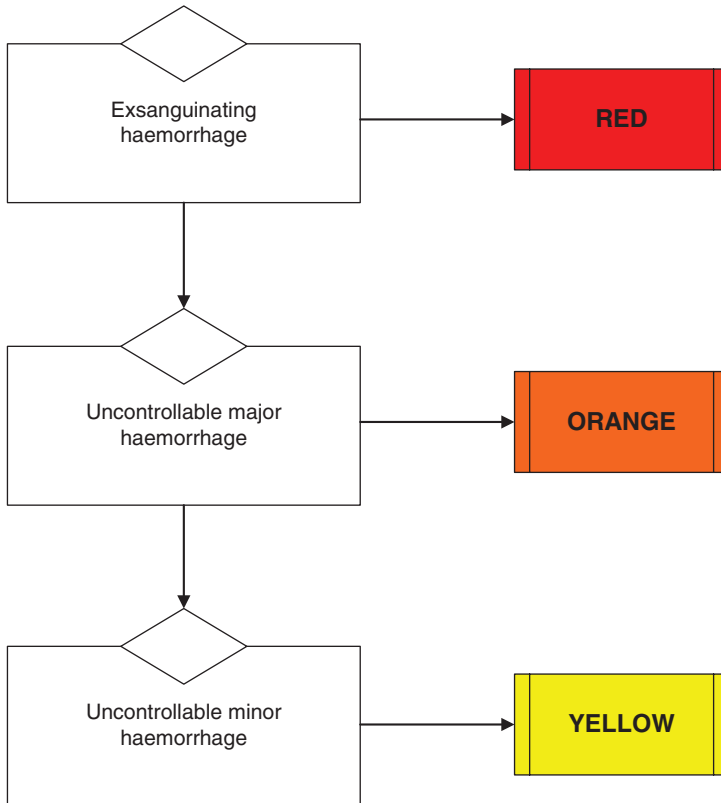




Haemorrhage

Haemorrhage is a feature of many presentations – particularly, but not exclusively, those involving trauma. The haemorrhage discriminators are exsanguinating, uncontrolled major or uncontrolled minor. The use of the success of attempts to control the haemorrhage is deliberate since, in general, continuing bleeding has a higher clinical priority. While, of course, in practice it can be difficult to decide which category a particular haemorrhage falls into, the definitions of the discriminators are designed to help the practitioner to do this. Exsanguinating haemorrhage is present if death will ensue rapidly unless bleeding is stopped. A haemorrhage that is not rapidly controlled by the application of sustained direct pressure, and in which blood continues to flow heavily or soak through large dressings quickly, is described as an uncontrollable major haemorrhage, while that in which blood continues to flow slightly or ooze, is described as uncontrollable minor haemorrhage.

Any bleeding, however minor, will, unless another discriminator leads to the allocation of a higher clinical priority, be allocated to priority 4 (green).



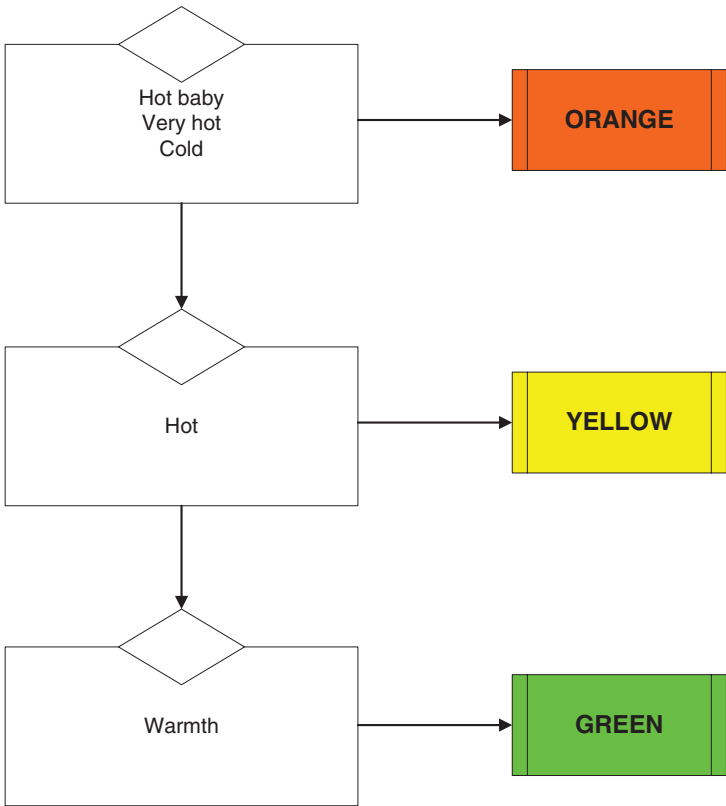
Temperature

Temperature is used as a general discriminator and accurate measurement of the temperature should be a part of the triage process, where indicated. Clinical impression of skin temperature is important and is crucial where immediate assessment of core temperature is not possible.

If the skin feels very hot, the patient is clinically said to be very hot – this corresponds to a temperature of 41°C or more; similarly if the skin feels hot the patient is clinically said to be hot and this corresponds to a temperature of 38.5°C or more. A patient with warm skin fulfils the discriminator of warmth and this goes with a temperature of less than 38.5°C .

Patients with cold skin can be said to be clinically cold – a core temperature of less than 35°C matches this.

A very hot patient (1 year and above) will always be categorised as priority 2 (orange), while a hot patient will be categorised as priority 3 (yellow). Patients who are cold will be allocated to priority 2 (orange). The hot baby (from birth to 12 months) will always be categorised as priority 2 (orange).

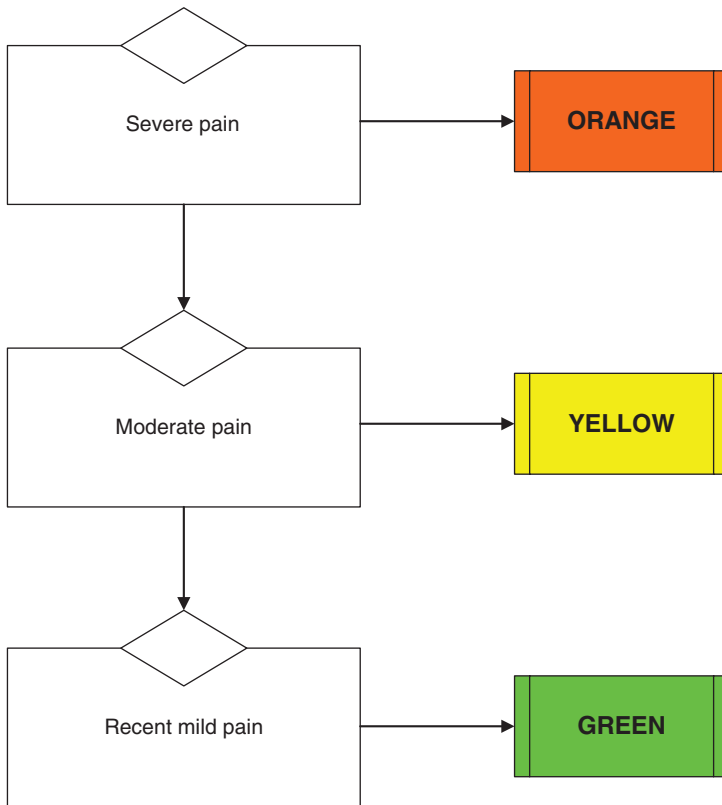


Pain

From the patient's perspective, pain is a major factor in determining priority. The use of pain as a general discriminator throughout the presentational flow charts recognises this fact and implies that every triage assessment should include an assessment of pain. Pain assessment is dealt with in chapter 4 and readers are referred there for a detailed discussion. In general terms, the discriminator severe pain is intended to imply pain that is unbearable, often described as the worst ever, while moderate pain refers

to pain that is bearable but intense. Any patient with a lesser degree of recent mild pain should, if no other discriminators suggest a higher categorisation, be allocated to priority 4 (green) and not to the non-urgent category, priority 5 (blue).

The general pain discriminator describes the intensity or severity of pain only. Other characteristics of pain, such as site, radiation and periodicity, may feature as specific discriminators in particular presentational flow charts.

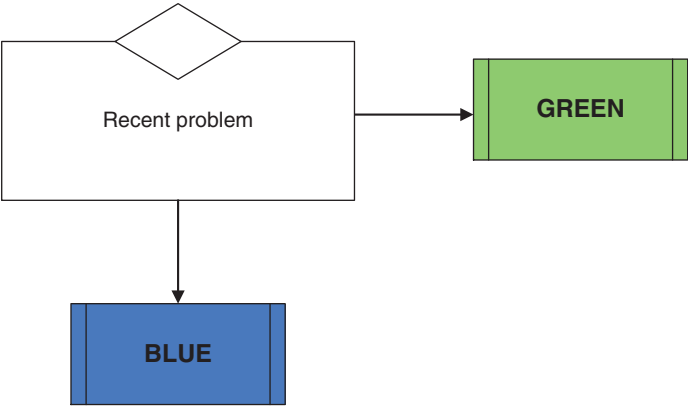


Acuteness

Within the triage method certain conventions have been used to help with consistency. The term 'abrupt' is used to indicate onset within seconds or minutes and 'acute' indicates a time period within 24 hours. Recent symptoms and signs are those that have appeared within the past 7 days.

Whilst most clinicians have no problem accepting that the acuteness of onset can help indicate the clinical priority, it is slightly more controversial to argue that chronicity (in this case greater than 7 days) is used to define a non-urgent problem. However, on reflection, it is intuitive that the relatively long time that the problem has been present indicates that the patient can be allocated the non-urgent priority without clinical risk. The triage method is such that the presence of any other general or specific discriminators relevant to the presentation will result in the allocation of a higher priority, for example, recent mild pain.

The use of this discriminator is not intended to ‘punish’ patients for turning up ‘inappropriately’, nor is it intended to ensure that patients who have had injuries or illnesses for a long time have extended waiting times. The actual waiting time for patients with stable problems not of recent onset will depend on the current case mix and case load of the department, and the resources available.



Secondary triage

It may not be possible to carry out all the assessments necessary at the initial triage encounter – this is particularly so if the workload of the department is high. In such circumstances the necessary assessments should still be carried out, but as secondary procedures by another member of the team. More time-consuming assessments (such as blood glucose estimation and peak flow measurement) are often left to the secondary stage. Many charts have a ‘risk limit’ placed on them, which indicates the lowest priority that can be applied to the patient if all observations needed are not complete.

Evaluating alternatives and selecting one

Selection of the most appropriate flow chart presents a number of general and specific discriminators which can then be tested against the patient. The skill in implementing the triage method lies in the application of this testing. Practitioners must decide whether the criteria for the presence of each discriminator is fulfilled, and must decide which discriminator is the most applicable at the highest clinical priority. For example, the patient who presents with neck pain following direct trauma to the neck and a pain score of 6 (both priority 3), the most appropriate discriminator is *direct trauma to the neck* as this provides more significant information about this patient.

Implementing the selected alternative

This step is essentially a procedural one. The inevitable outcome of the information gathering, analysis and evaluation leads to allocation of one of the clinical priorities shown in the Table 3.1.

Table 3.1

Number	Name	Colour
1	Immediate	Red
2	Very urgent	Orange
3	Urgent	Yellow
4	Standard	Green
5	Non-urgent	Blue

Documentation

Implementation involves recording the allocated priority and showing the decision making that led to it. The triage method outlined here allows documentation to be simple and precise. The minimum required is a record of which presentational flow chart is being used, which discriminator defines the category and which category has been selected. Thus, for instance, the triage record of a patient with chest pain might be:

Chest pain Pleuritic pain Urgent/priority 3 (yellow)

This simple approach to documentation allows for simple audit, but it is recognised that computer decision support software is widely used and will dictate the way the triage event is recorded.

Patient assessment

The purist view of the triage event is a rapid and focused encounter in which information is gathered and applied to assign a priority. This type of assessment is a skill in itself. The following framework can be used to teach the process to triage practitioners, ensuring decisions are based on relevant and appropriate patient data.

It is important that the assessment of a patient is systematic and all elements of that assessment are pieced together to give a complete picture of the patient’s problem. For this reason the triage practitioner should have sufficient experience of emergency care and the interpersonal skills to communicate effectively with patients and their families.

The approach to this assessment should take the following format (Table 3.2):

Table 3.2

Assessment component	Triage activity
Greeting the patient	The assessment begins at first sight of the patient; watch the patient as they approach the triage area, and pick up on any visual signs, which may include: • level of mobility • obvious injury • age of patient
Patient’s history	Ask the patient why they have attended the Emergency Department <i>This is a short, concise, subjective history and tells you about the patient’s injury/illness/health-related problem</i>
Presenting complaint	The patient’s presenting complaints can be established from the subjective history they provide <i>This leads the triage practitioner to choose the most appropriate presentation flow chart</i>
Focused questions (interview)	This is where the triage practitioner’s knowledge and skills are most evident. Application of anatomical knowledge, pattern recognition of presenting complaints and the ability to react effectively to life-threatening situations are all the domain of the triage practitioner

Table 3.2 (Continued)

Assessment component	Triage activity
	Focused questions can be used to obtain more detail if required, e.g. mechanism of injury, duration of the problem, current medications, etc. <i>The format of these questions will be directed by the discriminators in the chosen presentation flow chart</i>
Physical examination and assessment of physical parameters	If appropriate: • location of actual sites of injury • recording of baseline observations, pulse, temperature or more detailed information, e.g. obtained from pulse oximetry or assessment of visual acuity
Pain assessment	An integral part of the MTS, both subjective (patient) and objective (triage practitioner) pain scores are worth recording with documentation of the rationale for differing scores
Priority/plan of care	Priority assigned using the most appropriate discriminator applicable to the patient. Briefly describe any further care identified as a result of the triage assessment
Documentation	The recording of this information should be in an agreed format and should be clear, concise and relevant to the presenting complaint When a computerised triage system is in place, the triage practitioner should make sure the focus of attention is always the patient and not the computer screen/keyboard Include a record of any: • allergies • current medications • relevant past medical history • first aid measures applied at triage • observations • drugs administered, e.g. analgesia <i>Make sure signature is legible</i>
Reassessment	Document where there is a need to reassess, in particular when analgesia has been administered at triage

By following this systematic process, facilitated by the triage methodology, the patient assessment can be performed rapidly and confidently to reach an appropriate clinical priority in order to guide decision making.

Monitoring and evaluating

Clinical priority can change and triage must therefore be dynamic. The triage method described here can be carried out rapidly and reliably by qualified clinicians; it is therefore useful as a tool for multiple re-evaluations of clinical priority during the patient's stay.

Every encounter can be used as a triage assessment and any change in clinical priority can be rapidly notified and acted upon.